DR Care Solutions

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# Life Plan™

This Life Plan™ is completed for all care needs assessments undertaken by a consultant from DR Care Solutions. It's used as a guideline as to the care options available for a specific client(s). It is a central place to capture your preferences around future ageing related care, types of support and living arrangements.

CLIENTS NAME: **JOHN 'ZVONKO' FRISCIC**
DOB: **21.08.1942 (82 years of age)**
CLIENT ID: **DRCS 0530**

| **Issues covered:** | **Background and relevant history;** Bianca Friscic reached out to Danielle from DR Care Solutions on 27th November 2024 to discuss her father, John's ageing care and living needs. We undertook an initial TEAM's meeting with Bianca discussing how John's condition was altering and how she is managing his care and support currently.<br><br>We arranged to meet with Bianca, John and Bianca's brother, Troy at John's home in Mosman on Tuesday 21st January 2025 with the main aim to assess his current and future care needs and develop a plan to support John moving forward. Bianca, John and Troy welcomed Danielle into the home. The assessment took just under two hours.<br><br>Bianca was referred to Danielle by her Financial Adviser, Melinda Measday of HLB Mann Judd.<br><br>John has had significant health issues since April 2022 when he was admitted to hospital with sepsis of his Gall Bladder and gangrene of the Gall Bladder. He went into organ failure and was very unwell. He spent 10 days in the ICU (Intensive Care Unit) and 10 days in the Acute Care Ward at Royal North Shore Hospital. He was then transferred to North Shore Private Hospital for multidisciplinary rehabilitation. He was seen by Dr Anita Warrier (Geriatrician) while he was there, and she diagnosed him with Vascular Dementia. Based on follow up assessments and appointments, he has since had a further 'possible' diagnosis of Alzheimer's disease. He had a few falls in hospital. More recently, he fractured his right wrist and hurt his right shoulder in a fall at home.<br><br>John developed Atrial Fibrillation. He remains very mobile as there is three to four flights of stairs internally at home. He rarely leaves the home now (since he has stopped driving). |
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| • Physical health including mobility, continence,<br>• Cognitive health<br>• Sensory (sight, hearing, oral, feet)<br>• Activities of daily living<br>• Risk assessment of the home and modification recommendations<br>• Medications, allergies & vaccinations<br>• External assistance - including Allied Health<br>• Emotional & mental health<br>• Social background, networks and Relationships | |

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| • Likes & dislikes<br>• Cultural linguistic and Spiritual needs<br>• Estate Plan/ Advice and Life Ready Register<br>• Financial Plan/ Advice<br>• My Aged Care<br>• Care and support considerations | John can climb the stairs safely and holds onto the rail. There is a chair lift installed from the first to second floors. This was installed for John's wife, Nina before she passed away. He advised he doesn't use it as he doesn't need to. While ambulating, we recommended to John that he wears closed in shoes rather 'slip on' shoes as these can often cause falls for older Australians.<br><br>John smoked many years ago (1960). He no longer smokes. He enjoys an occasional 'black' beer or a glass of champagne.<br><br>**Current health, cognition and physical (mobility) condition**; Physically, John appears well. Cognitively, John's short-term memory is rapidly deteriorating. He mentioned he feels dizzy and foggy at times. From these symptoms, it was determined he should no longer be driving. He also has been getting lost in his local area and unaware of his need to drink fluids on hot days. John's conversation is often rambling. His judgement and insight remain impaired.<br><br>John remains vulnerable to exploitation. He's been deemed to have lost capacity and Bianca and Troy have enacted their Powers of Attorney (for John's financial decisions) and Enduring Guardianship (for John's health, care and accommodation decisions).<br><br>John is hallucinating. Bianca and Troy mentioned that he is speaking to people who aren't there. He's confusing stories on the news for real life events that he's involved with ie: he thought a young girl was missing in his area and he was sure he needed to help find her.<br><br>John tends to put things away or hides things and then can't locate them. Bianca mentioned it's like a scavenger hunt to locate things in the home. He's misplaced his iPad and Apple watch. They haven't yet been found.<br><br>**Clinical needs;** John's skin is intact however he's on blood thinners and bleeds profusely when he cuts himself. He advised he uses Teatree oil on cuts which helps heal them. He forgets to take his medications; even though they are in a Webster-Pak®. John is continent of bowel and bladder. He has some urgency of micturition yet, no bladder accidents to date. He doesn't wear pull up pants currently.<br><br>**Sensory needs:** John has had two cataracts removed, one in 2022 and one in 2024. He wears glasses for reading. He sees the dentist annually and has all his own teeth with a few teeth implants. He has his nails (fingers and toes) trimmed at a beautician. He really enjoys this activity on a regular basis. We suggest he is seen by a professional podiatrist to ensure not only good nail and foot health; however, they can check his balance and need for orthotics for fall prevention. |

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| | **Activities of daily living;** John advised he can manage all his showering, dressing and undressing. He was clear he doesn't need to shower 'every day'. It might be a good idea when a carer is present that John showers then. They can provide standby supervision. He doesn't want to use a shower chair.<br><br>John doesn't use the gas cooker (he's not comfortable or confident using the gas stove). He uses the oven with a timer. Bianca prepares food for him which he just needs to heat up. John's weight is stable. He's not under nor overweight.<br><br>John suffers from constipation at times. He enjoys eating fruit including berries and drinking coffee (short black). He may require Sustagen smoothies with fruit to maintain his protein intake. Prunes / prune juice may assist naturally with any constipation issues.<br><br>John wakes around 8.30am and falls asleep on the lounge at around 8pm. John stated he has no problems sleeping until 2am, he wakes up on the lounge and takes himself to bed.<br><br>John enjoys sweeping the pathways around the pool area. A gardener is required to trim, maintain and tidy up the garden areas. There are no lawns requiring mowing. There is a pool needing maintenance.<br><br>**Emotional and mental health;** John is lonely and socially isolated, now that he can't drive. He is reliant on Bianca (who lives next door) and Troy who lives in Waterloo to transport and accompany John to wherever he needs to go. John was previously going into work with Bianca (family business) on a regular basis. He was able to leave (drive home) when he'd had enough. Since he's ceased driving, this is problematic as Bianca needs to transport him home. Some days John doesn't feel like going into work and remains at home alone.<br><br>At times, John becomes anxious and disorientated.<br><br>**Risk assessment of the home and modification recommendations:** We suggest that an OT (Occupational Therapist) assess the home for risks and modifications to ensure it's safe for the long-term care of John. It's set on a rock face in Mosman with three to four flights of stairs from the street to the top living area. It's a large home with a swimming pool. John mentioned he enjoys going into the pool and uses a 'noodle' to float. Danielle suggested he only go into the pool when someone else is present, as a precaution. He fell into the pool while cleaning it around 6 months ago.<br><br>The home is large with a chair lift over two levels. John doesn't believe he needs to use it. An internal lift could be installed if John's mobility became compromised. |

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| | Every room had items, clothes, books and boxes in it. There are two bathrooms (that we saw), and the shower has a small hob to enter. It is suggested that John only showers when a carer (or one of his children) is present. There is a spare room and bathroom for live in carers if it's needed in the future.<br><br>**External assistance**: Bianca explained she had engaged some gardeners recently who over charged her for the time they were present. She has cameras on the outside and inside of the home. She will need to source a new Gardening firm. Danielle suggested the home care provider we can arrange will have relationships with gardening firms or we can source a provider for them.<br><br>A pool cleaner / maintenance person has been engaged. His name is Gabriel and he comes fortnightly to provide a maintenance service.<br><br>There is currently no external cleaning provided.<br><br>**Medications, Allergies and vaccinations:** John is not immunised. He remains unvaccinated for Covid, Flu or Shingles. We don't believe that John is allergic to any foods or medications. From the Webster-Pak®, we are aware of John's current medications. **It is also noted that he is not taking his medications on the required days and times.** Supervision and administering medications will be needed moving forward.<br><br>John's current medications include:<br><br>• Apixaban (Blood thinner to lower the risk of blood clots & stroke)<br>• Macuvision (for maintaining healthy eyes)<br>• Metformin (for managing Type 2 diabetes)<br>• Mirtazapine (an anti-depressant to treat major depressive disorders)<br>• Rosuvastatin (to treat high cholesterol and prevent heart attacks and strokes).<br><br>**Social background, networks and relationships;** John was born in Zagreb, Croatia. He had a younger brother, Sylvester. The history (during war time) is a bit patchy. The family went via Germany by ship to Australia in 1957. They landed in Albury at a property called Bonegilla in Victoria which was a migrant town. His dad was working in the mines initially and then made suits in a factory.<br><br>John met Nina and they married in around 1970 / 1971. John became a performer; he was a juggler, hypnotist, Punch and Judy puppeteer and ran 'merry go rounds.' |

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| | Nina felt they needed a more stable income once they were married (around 1971). John became a salesman selling cash registers, catering equipment and microwaves from the back of his car.<br><br>John and the family started their own business and were the sole importer and distributor of Ily Coffee in Australia from 1996 to 2015. John would attend the office 6 days a week (Monday to Saturday). Since 2015, they're no longer importing the coffee, however they distribute it and manage their assets. Bianca's main role is administration and Troy's is the front end of the business.<br><br>John's wife, Nina, passed away from cancer (liver and bone cancer) in October 2018. She was a formidable force who helped set up the business and kept the family intact. Since her passing, there has been a huge void. John has become increasingly reliant on Troy and Bianca to run the business (and ensure he is well cared for).<br><br>They've had some staffing issues since Nina passed away, where an ex-employee attempted to exploit John by wanting to become his carer pushing Bianca and Troy away. This has only recently been resolved by the law and court injunctions.<br><br>Sylvester, John's brother passed away last year in Croatia. He had cochlear implant surgery, a heart bypass and a stroke. The family are unsure of what exactly happened; however, he was very unwell before passing. Sylvester's wife remains in Croatia.<br><br>John used to be very good at accounting and numbers. His ability to understand the business financials has diminished.<br><br>Troy was born in 1971 (54 years) and Bianca in 1974 (50 years).<br><br>Troy has a partner, and they have a son, Charlie (15 years). He lives with Troy in Waterloo.<br><br>Bianca is married to Bill. They have a daughter and a son. Nina (19 years) is studying an Advanced Diploma of Interior Design and Lucas (15 years) is at Redlands High School in year 10. He's heading to Winter School this year and is a very keen and talented basketballer.<br><br>Troy's son, Charlie and Bianca's son Lucas are close and spend time together at the office playing the drums and guitars during their holidays.<br><br>The family built the house in Mosman after demolishing a smaller, older home. The family moved into the home in 1990. Bianca purchased a house with her husband next door to John.<br><br>John's social network is made up mainly of family, however he has a mate from work, Joe who keeps in touch with him on a regular basis. |

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| | **Likes and dislikes:** John used to travel a lot between Sydney, Melbourne and Brisbane for his work. He can no longer do this as he was very unsettled thinking he was in one location and was in another. He used to enjoy traveling overseas. He was meant to go to Croatia in 2020 however Covid intervened.<br><br>John loves visits from his grandchildren. John enjoys going to the City and Surry Hills areas and being part of the community. Trips on the Metro might be something he would enjoy.<br><br>Electric guitars and a drum kit is set up at the Mascot office. He loved listening to records of rhythm and jazz music. His grandsons enjoy visiting the Mascot office during school holidays to play the instruments.<br><br>John loves Fish & Chips and in fact he's a good eater of all foods.<br><br>John spends much of his time watching Instagram and YouTube reels. He's been watching a bit of the tennis and television. He watches the news and current affairs.<br><br>Currently, John hasn't much occupation, socialisation and is bored. He is stubborn and is likely to refuse support even if it will enhance his quality of life. We mentioned we could arrange a carer daily to take him on interesting outings and activities of interest to enhance his quality of life. It will take a special carer(s) to engage him to get out and about in the community.<br><br>**Cultural, linguistic and spiritual needs;** John can speak Croatian, English and a bit of German. He has been reverting to more and more Croatian words when articulating to Bianca and Troy. They can't speak Croatian and try to decipher what he's saying.<br><br>While John was born catholic, he only went to church for Christmas and Weddings. He has no requirement to attend church however this may change in time.<br><br>**Personal alarms and monitoring;** John previously had an Apple Watch which has gone missing, and it has run out of charge. The 'Find my watch App' isn't locating where John left it. He also has an iPad which has also gone missing. He believes he may have left them at the Office in Mascot. Apparently, John hides items and it's like a scavenger hunt to track things down.<br><br>We strongly suggest that if the Apple Watch can't be located, a new one is purchased and set up with GPS tracking and falls detection. The five contacts listed should include Bianca and Troy as well as emergency services. The watch should be always worn particularly in the shower and overnight (where most falls and accidents occur). |

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| | The watch can be charged when John either has a carer or someone else present in the home. In time, if John has live-in care at home, a baby monitor (video camera) can be set up in the carers room as well as John's room, so they are aware and alert if John is up and about during the night.<br><br>**DR Care Solutions – Spring Cleaning Right Sizing Guide:** We offer all DR Care Solutions clients a copy of our decluttering and downsizing guide. John's home is extremely large and there are a lot of items, papers, boxes throughout the home. Here is the link to our guide for your consideration. Decluttering or Downsizing? Being Organised Is Key<br><br>**Estate planning;** We have received e-copies of the Powers of Attorney documents (for financial decisions) and Enduring Guardianship documents (for health, care and accommodation decisions) dated 2016. They may have been reviewed by lawyers since that time.<br><br>We understand that Bianca is listed as Power of Attorney and Enduring Guardian for John and that he's been deemed to have lost capacity to make appropriate decisions by Dr Warrier. These documents have been enacted, and Bianca with Troy's assistance are managing John's finances, health, care and accommodation decisions.<br><br>**Advanced Care Directive:** Below we include a special DR Care Solutions link to Touchstone Life Care for building an Advanced Care Directive. You are welcome to use this or ask your GP for a paper-based document if it's preferred. This is usually stored in the client's GP Health Summary and hospital records should something occur, the medical profession adheres to the requests if the person is unable to communicate their wants, needs and wishes. Touchstone Life Care - A DR Care Solutions Partner<br><br>**Life Ready Register:** For DR Care Solutions clients, we provide a copy of the Life Ready Register, a depository to keep all your affairs in one place. We suggest that you download this and start completing it. No matter what age, everyone can commence building this out and sharing it with their trusted network, their Powers of Attorney and Enduring Guardians as well as their Executor of their Will. We often find the adult children of our clients using this document. Life Ready RegisterTM - Free Aged & Disability Care Industry Resource<br><br>**Funeral arrangements:** We suggest considering organising Funerals well before it's required. We work very closely with Funeral Directors (from the Family Business Sector) and would be more than happy to introduce you to businesses that cover your area if requested. Be mindful of payment plans when it comes to funerals, as often they cost more than a standard funeral. |

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| | Preplanning eases your family's burden during an emotionally difficult time. If you have specific questions, feel free to ask.<br><br>**Financial planning;** John is a self-funded retiree. This doesn't mean that he can't access Government Funded Services. Some services are not means tested (Commonwealth Home Support Program which is entry level access to services within My Aged Care) and others are means tested (Home Care Packages, Residential respite care and permanent residential care). For more information on how the Government Support works, please review this section on our website. Government Support \| DR Care Solutions.<br><br>**My Aged Care;** Bianca has attempted to register with My Aged Care online, without success. She was under the impression that John wouldn't be entitled to any Government Funded care or support.<br><br>We suggest that Bianca phones My Aged Care (following the steps in our easy-to-follow Aged Care Puzzle).Government Support \| DR Care Solutions.<br><br>An ACAT assessment is required. Bianca needs to be transparent with John's current condition and hers and Troy's involvement in his day-to-day care and support. Please ensure you ask for an assessment and referral codes for **a) a Home Care Package (which is means tested), b) Residential Respite Care and c) Residential Permanent Care (both are means tested).** The codes for residential care will unlock the ability for John to enter a supported living environment if / when the time comes.<br><br>**Care and Support considerations:** There are several services we can commence immediately if requested.<br><br>We suggest a **cleaning company** be engaged to come in to undertake a **spring clean of the home** (may need **two cleaners for a full day**) and then a regular ongoing **maintenance service of the home** either weekly or fortnightly **(for 4 – 6 hours).** We can make a referral to a preferred provider on the DR Care Solutions, preferred provider network to quote and undertake the cleaning.<br><br>We suggest a **steam clean of the carpets** is also necessary to bring the home up to a maintainable level. The home needs to be aired.<br><br>We suggest that a suitably skilled carer(s) is engaged to be with John for a **6 to 8 hours each Monday to Friday** to provide him with structure, outings and activities as well as ensuring his medications are taken at the appropriate time, that he showers safely, he is hydrated appropriately and provided with companionship. |

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| | Bianca discussed with Danielle to start slowly with **three days a week** for 6 – 8 hours each visit to see how John engages with a male carer (Monday, Wednesday and Friday) with the view of increasing the days and hours over time. It will be a matter of the carer having initiative and advising John that "today we are doing …" rather than asking John what he wants to do. If he says he doesn't wish to go out, then activities at home, sorting through rooms might be something a carer could support John to do.<br><br>John enjoys going to the city area and being part of the community. Trips on the Metro might be something he would enjoy. Outings to beaches, parks, exhibitions, galleries would support daily occupation for John. The carers could also take him across to Mascot to the office for a visit if he wanted to do that. Everyone should avoid discussions around John driving. His medical practitioners have advised that he should no longer drive for his own safety and others.<br><br>Morning tea and lunch out would really enhance his quality of life; however, they could prepare a fresh meal at home if John didn't feel like going out into the community and ensure he had dinner prepared and set up for him. This would take some of the pressure of Bianca and Troy. The carers would tidy the areas where John spends most of his time, clean the kitchen after meal preparation, ensure the bathroom is clean and dry after his shower. They could plan 'together' what the carer would do the next day or next time they will be together. |
| **Goals** (what does the client wish to achieve):<br><br>● Options<br>● Preferences<br>● Ideal Outcome<br>● Carer Profile | John (Bianca and Troy) expressed the wish for John to **remain at home** with quality care and support in place. They didn't know if this was feasible or available. It is. We explained that getting a quality home care provider (with a Case Manager overseeing their care) having a supportive team around him to ramp up or scale down the hours and days, 'as needed' is what needs to occur.<br><br>As an **alternate option**, John could move into residential aged care for a period of respite (usually a minimum of three weeks). This can be disruptive to John's routine and could escalate the progression of his dementia if he's then moved back home. He could enter for a period of respite and the family may make the decision for him to remain there permanently. We've seen older Australian's thrive in a supported living environment.<br><br>As John has the capital resources to remain at home with 24-hour care, the family should be given that choice. Care and support can be provided in the home until the end of life if it's realistic and financially viable. We have several clients around Australia who have 24-hour+ care in place. |

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| | **Carer Profile:** We envisage strong, caring, empathetic and highly skilled male carers be employed to support John to remain at home for the long term. Intelligent, well-travelled and educated carers, who can converse with him would be ideal. Carers who can speak Croatian would be an added plus however it's not essential.<br><br>A care team could be gradually built up over time. A team of 2 or more carers (male carers) would be a great support to John, 'as needed' physically, emotionally, and intellectually.<br><br>Carers from the local area would be preferred and with a clean driver's license and a well-maintained car will be essential.<br><br>Homemaking skills would be advantageous to maintain the home including changing the bedlinen, undertaking the washing and laundry, cleaning and drying the bathroom after showering yet have the necessary 'carer' skills to help John in the shower, dressing, undressing, toilet, bed, transfers 'as needed'. Assistance with shopping and carrying heavy groceries will be required as well as meal preparation and cooking of healthy nutritious meals.<br><br>Carers who have clinical knowledge of someone living with dementia who can identify any changes in John's condition would be helpful to relay back to their Case Manager (Generally a Registered Nurse).<br><br>The carers need to give John the space he needs when he needs it and provide him with encouragement to get out into the community as they see fit. They need to be able to plan activities of interest for John and use their initiative. We expect any carers re-direct John from watching television or YouTube and undertake other activities.<br><br>Bianca advised that she would arrange a debit card (with a small amount of funds) for the carers to be able to take John out into the community for morning teas, lunches and activities. Receipts will be kept and provided to Bianca after each shift. |
| **Actions required:**<br><br>(for the client to achieve these goals) | If the decision is to engage support at home, including the provision of carers for John, then the immediate focus is to get this support in place. If there is resistance from John, he has **two choices**, care and support at home OR care in a supported environment to ensure he is safe and well cared for.<br><br>1. **Consider arranging immediate in-home care and support.** We can obtain quotes from providers as a comparison. We will need estimated days and times that John is considering initially. Please note our suggestion is **6 – 8 hours a day,** Monday to Friday. Bianca would like to trial **three days initially** (Monday, Wednesday and Friday) with a view to increasing days over time. |

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| | 2. Consider arranging **a spring clean of the home** with ongoing **maintenance cleaning** weekly or fortnightly. We can arrange a quotation for this service from a provider on our preferred provider network.<br><br>3. Consider arranging a **steam clean of all the carpets** throughout the home to bring them up to maintainable level (using a local provider) on an annual basis is suggested.<br><br>4. Continue with **pool maintenance and cleaning service** on a fortnightly basis.<br><br>5. Arrange a **gardening service** to trim and tidy the garden on a fortnightly or monthly basis. We can obtain quotes from providers for you.<br><br>6. Request an ACAT assessment (from My Aged Care). As part of the ACAT assessments, please obtain referral codes for **a Home Care Package as well as Residential Care (respite and permanent)**. Even though there are long wait lists for the assessments and for the funds to be released, we suggest getting into the system sooner rather than later. Similar to an insurance policy, better to have the codes and not need them, than need them and not have them (particularly for residential aged care).<br><br>7. Annual review of the existing **Estate Planning Documents** including wills. Establish back up Powers of Attorneys, Enduring Guardians and Executors.<br><br>8. **Development of an Advanced Care Directive with** specific emphasis on healthcare and accommodation decisions. It may be too late for John to make the decision; however, Bianca and Troy can do this together on behalf of their father.<br><br>9. **Consider completing the DR Care Solutions, Life Ready Register** and provide the completed document to their Next of Kin and Executors. Information provided in this life plan™.<br><br>10. Consider setting up a **prepaid funeral** in event of John passing. Please advise Danielle for an introduction to one of our Strategic Partners (Family Business) if interested.<br><br>11. The use of **personal alarms** or monitoring devices for emergencies such as falls. Consider purchasing a new Apple Watch (with GPS tracking and Falls Detection) if the old one can't be located. Carers to charge it when they arrive each day and the setup of five emergency contacts including Emergency Services. |

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| | 12. John to have a **hearing test** and obtain hearing aids if deemed necessary.<br><br>13. John to have his **eyes tested regularly** and obtain new glasses if deemed necessary.<br><br>14. Regular visits to the **dentist,** six monthly to ensure he maintains good oral health.<br><br>15. John to visit a professional **Podiatrist** to maintain foot health, nail health and aid balance (Check with GP as may be able to obtain five free visits per annum via the GP Health Plan).<br><br>16. Regular visits to the **GP and Geriatrician** as John's condition progresses. |
| **Planning ahead and Implementation:** | **Planning ahead:** Should John's condition deteriorate to a point where he requires care and assistance, 24 hours a day, 7 days a week, then the family can 'ramp' up care at home either with one live in carer on site, 24 hours a day or rotating shifts of two carers each doing 12 hour shifts or three carers per day each undertaking an 8 hour shift.<br><br>Financially, if John can sustain 24 /7 care at home in the long term then this is a good option. This would need to be confirmed by John's Financial Advice team, and we would work with the family to determine if this is the right option for John at that time.<br><br>If it's not viable for him to remain at home, then alternatively, we (at DR Care Solutions) could look to relocate John quickly and professionally into an assisted / supported living environment (Residential Aged Care) or a group home environment to ensure he is safe and well cared for and has social connection, particularly if John's behaviour or physical condition deteriorates significantly (with double incontinence developing, continual falls or wandering from the home or aggressive behaviours).<br><br>We recommend that anyone over the age of 80 years only has ONE move from their own home as it's stressful at the best of times, let alone being over 80. It can dramatically increase the progression of any cognitive disease if they need to relocate more than once (ie: from home, into a retirement community into residential aged care).<br><br>**Implementation:** When the decision is made to implement this plan, we suggest that DR Care Solutions is engaged to select and introduce a suitable approved, pre-vetted Home Care Provider to support John to remain at home. We will introduce **up to three** pre-vetted quality home care providers (approved by the Government). |

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| | One provider may have the perfect carers and support for John and therefore only one provider will be introduced. We need to ensure both his care needs are being met including occupation, physical activity, community access and socialisation.<br><br>Consistency of carers is key to ensure both feel safe and comfortable. A team of at least **two skilled male carers** should be able to cover the initial 3 - 5 weekdays needed. We always like to have at least two carers in a roster to ensure there is always someone who knows the client's routine and where things are kept, particularly if one carer is unwell or unavailable.<br><br>A Case Manager is generally allocated to the family from the Home Care Provider, and they will visit them regularly to ensure there is complete oversight with the team of carers and ensure that they have the appropriate support at that time. A Case Manager is generally a Registered Nurse who can clinically quickly pick up any issues in relation to their condition, behaviour or clinical care needs.<br><br>The rates being charged by providers 'fee for service' **range from $70 to $88 per hour** Monday to Friday for fully insured and employed carers (Minimum charge is generally 3 hours per visit). Weekends and public holidays incur surcharges.<br><br>Review of the hours and John's situation should be undertaken monthly by the Case Manager and relayed back to the family to ensure the right amount of care and support services are taking place for him at any given time. Government funding can be added to the mix of funding once it comes through. Home Care packages through the Government are means tested.<br><br>The goal is to keep John at home for the long term. We need to ensure John's physical and cognitive wellbeing is adequately taken care of to enhance his quality of life.<br><br>Some providers offer a LIVE-IN service (24 hours a day) with one carer per day. The client needs to be assessed (by the provider) as to whether they need support overnight or just need someone on site in case of emergency. We can discuss this option, if or when it is required. Other providers only offer coverage of the 24 hours in 8 hour or 12 hour rotating shifts. We will guide and support you through this process to get the right outcome for John.<br><br>As outlined above, should John's condition deteriorate significantly to a point where he can't be cared for at home safely, we can be **re-engaged** to find the right Aged Care Home or Group Home for respite with a view to permanent care. We run a comparative platform report bringing up the top 10 Aged Care homes in a certain area. |

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| | We work closely with the family to find the right home for the client and tour up to four or five in a particular location. It's not just locating a bed for a client, it's their next home. We personally ensure that the selected home is appropriate for John's needs, wants and wishes within his financial capacity to afford the home and that he is clinically accepted by the home. It's not just the family determining the home<br><br>We undertake all the groundwork, pre-vetting and due diligence of the aged care homes and remove the pressure and stress out of finding the right home at an already emotionally charged time. We walk alongside you for as long as John needs care or is in care. |
| **Resources:** | Explore Aged Care Puzzle, a resource designed to guide you through every step of the aged care journey. With our step-by-step puzzle solution, you can confidently make informed choices at your own pace.<br><br>Just pick your first Aged Care Puzzle piece, and you're on your way!<br><br>Aged Care Puzzle \| DR Care Solutions \| DR Care Solutions |
| **Who will be involved:** (Guardian, solicitor, financial planner/advisers, family members) | • Bianca and Troy Friscic<br>• Danielle Robertson from DR Care Solutions<br>• Melinda Measday, Financial adviser, HLB Mann Judd<br>• Family Accountant<br>• HWL Ebsworth - Estate Lawyers (Brisbane, Sydney, Melbourne)<br>• GP, Geriatrician, and any other medical specialists |
| **Review of Life Plan and the date the Family need to determine next steps:** | Thursday, 30 January 2025 |

Signature (Consultant):  Date: Monday 27th January 2025

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